Mante Inc Medical Center

1429 Oak Ave, Hartford, CT 06106
Phone: (804) 885-2668 | Fax: (207) 396-3996

DISCHARGE SUMMARY

PATIENT INFORMATION

Patient Name: Kirstin Smith
Date of Birth: 12/21/1994
Admission Date: 04/11/2026
Attending Physician: Dr. Sylvan Nicolas

MRN: MRN-47763276
Insurance ID: INS-7542120567
Discharge Date: 04/16/2026

VITALS AT DISCHARGE

BP: 108/75 mmHg
Temp: 100.0 F
Resp Rate: 19 breaths/min

HR: 79 bpm
SpO2: 98%
Weight: 204 lbs

DIAGNOSES

1. Gastroesophageal Reflux Disease (K21.0)
2. Chronic Obstructive Pulmonary Disease (J44.1)
3. Atrial Fibrillation (I48.91)
4. Anxiety Disorder, unspecified (F41.9)

HOSPITAL COURSE

Patient was admitted via the emergency department following acute onset confusion and altered mental status. Repeat laboratory values showed improvement in renal function and resolution of leukocytosis. Pulmonology was consulted and recommended pulmonary function testing as an outpatient. Physical therapy and occupational therapy were consulted for early mobilization and functional assessment. The patient's pain was adequately controlled with scheduled acetaminophen and PRN opioids. Wound site was assessed daily with no signs of erythema, warmth, or purulent drainage. Nutrition services were consulted for optimization of caloric intake and supplementation. Repeat imaging demonstrated interval improvement with no new findings of concern.

Patient was admitted via the emergency department following hypertensive urgency with headache and visual changes. IV access was established and aggressive fluid resuscitation was initiated. DVT prophylaxis with subcutaneous heparin was maintained throughout the hospitalization. Respiratory therapy provided nebulization treatments every four hours with improvement in oxygen requirements. Home medications were reviewed and held or adjusted as clinically indicated. CT imaging of the chest and abdomen was obtained and reviewed with radiology. Repeat laboratory values showed improvement in renal function and resolution of leukocytosis. The patient was educated on fall prevention and instructed to use the call light as needed.

Patient was admitted electively for management and optimization of chronic conditions. Cardiology was consulted and recommended rate control and anticoagulation therapy. Physical therapy and occupational therapy were consulted for early mobilization and functional assessment. Patient was admitted for acute management and close hemodynamic monitoring. CT imaging of the chest and abdomen was obtained and reviewed with radiology.

Patient presented with a 2-day history of bilateral lower extremity edema and was admitted for further evaluation. DVT prophylaxis with subcutaneous heparin was maintained throughout the hospitalization. Repeat laboratory values showed improvement in renal function and resolution of leukocytosis. Initial assessment revealed tachycardia and elevated inflammatory markers. Nephrology was consulted for acute kidney injury with recommendations to hold nephrotoxic agents. The patient's pain was adequately controlled with scheduled acetaminophen and PRN opioids. Nutrition services were consulted for optimization of caloric intake and supplementation. Strict fluid balance was maintained with daily weight checks and accurate intake/output recording.

Patient was admitted via the emergency department following acute onset confusion and altered mental status. CT imaging of the chest and abdomen was obtained and reviewed with radiology. The patient was placed on continuous telemetry and pulse oximetry. On admission, the patient was alert and oriented, in mild-to-moderate distress. The patient's electrolytes were repleted as needed with close monitoring of serum levels. Blood cultures, urine cultures, and respiratory cultures were obtained prior to antibiotic initiation. The patient's pain was adequately controlled with scheduled acetaminophen and PRN opioids.

Patient was admitted electively for management and optimization of chronic conditions. Social work was involved to assist with discharge planning and community resource coordination. The patient demonstrated good understanding of the medication changes and dietary restrictions explained. The patient was empirically started on broad-spectrum IV antibiotics pending culture results. Case management coordinated a home health referral for post-discharge wound care and medication compliance. Cardiology was consulted and recommended rate control and anticoagulation therapy.

Patient was admitted via the emergency department following acute decompensation of a known chronic condition. By hospital day five, the patient met discharge criteria with stable vitals and tolerating oral intake. IV access was established and aggressive fluid resuscitation was initiated. Blood cultures, urine cultures, and respiratory cultures were obtained prior to antibiotic initiation. The patient was placed on continuous telemetry and pulse oximetry. The patient was educated on fall prevention and instructed to use the call light as needed.

DISCHARGE MEDICATIONS

1. Omeprazole 20mg - once daily before breakfast
2. Sertraline 50mg - once daily
3. Levothyroxine 75mcg - once daily on empty stomach
4. Hydrochlorothiazide 25mg - once daily
5. Amoxicillin 500mg - three times daily for 10 days
6. Atorvastatin 20mg - once daily at bedtime
7. Metoprolol 25mg - twice daily
8. Prednisone 10mg - taper over 7 days
9. Amlodipine 5mg - once daily
10. Metformin 500mg - twice daily

PROCEDURES PERFORMED

1. Bronchoscopy with lavage
2. Echocardiogram (transthoracic)
3. CT scan of chest/abdomen/pelvis with IV contrast
4. Blood transfusion (2 units packed RBCs)
5. Arterial blood gas analysis

CONSULTATION NOTES

Nephrology - Dr. Maurice Reynolds

Patient assessed for acute kidney injury, likely contrast-induced in etiology. Outpatient nephrology follow-up arranged with repeat metabolic panel in one week. Creatinine trending downward following fluid resuscitation and removal of offending agents. Chronic kidney disease staging reviewed; dietary counseling with nephrology dietitian recommended.

Pulmonology - Dr. Lucius Bogisich

Recommend continuation of broad-spectrum antibiotics and reassessment in 48 hours. Inhaled bronchodilators and corticosteroids initiated with good subjective response. Chest X-ray reviewed demonstrating bilateral infiltrates consistent with the clinical picture. Oxygen weaned to 3 L/min by day of discharge; patient instructed on home oxygen use.

Infectious Disease - Dr. Willa Windler

Blood cultures finalized with no growth at 72 hours; IV antibiotics de-escalated accordingly. Patient at risk for Clostridioides difficile; probiotics recommended and stool for C. diff sent. Patient evaluated for complicated infection requiring IV antibiotics. Source control assessed; no surgical intervention indicated at this time.

PATIENT EDUCATION & DISCHARGE INSTRUCTIONS

- Diet: Low sodium (<2g/day), diabetic diet as previously prescribed.
- Activity: No heavy lifting >10 lbs for 2 weeks. Gradual return to normal activity.
- Wound care: Keep incision site clean and dry. Change dressing daily.
- Warning signs: Return to ER if experiencing chest pain, shortness of breath, fever >101.5F, or worsening symptoms.
- Medications: Take all medications as prescribed. Do not skip doses.
- Follow-up labs: CBC, CMP, and coagulation panel in 1 week at Paucek - Kuhic Lab.
- Smoking cessation counseling provided. Patient advised to quit smoking.
- Home health: Gutkowski Inc Home Health will contact within 48 hours for wound care visits.

FOLLOW-UP INSTRUCTIONS

Follow up with Dr. Sylvan Nicolas on 05/06/2026. All follow-up laboratory work has been pre-ordered and scheduled at the outpatient facility. The patient should avoid strenuous physical activity until cleared at the follow-up appointment.

Electronically signed by Dr. Sylvan Nicolas
Date: 04/16/2026 05:57 PM